

Registration / Demographics

Patient Name:	Emily Clark	DOB:	1976-
Age:	47	Sex:	Female
Address:	345 Bendwood Lane, Springfield, IL 62701		
Phone:	(217) 555-0134	MRN:	93384
Insurance:	SynthCare Classic 250	Emergency Contact:	John C 555-9
Encounter Date:	2023-09-14	Check-in Time:	07:50
Location:	EGD Suite 2, Lakeside Gastroenterology Clinic		
Attending Gastroenterologist:	Dr. Connor Jonathan, MD	Referring Provider:	Dr. Ma
Chief Complaint / Indication:	Dyspepsia, recurrent upper abdominal pain. Suspected peptic ulcer disease.		

Medical History

PMH: Hyperlipidemia, hypertension.

PSH: Appendectomy at age 15.

FHX: Mother with GERD and gout, Father with hypertensive heart disease.

SHX: Non-smoker, social alcohol usage (~2 drinks/week). Possible prior selective NSAID u

Allergies: Penicillin (rash).

Problem List

- Periodical abdominal discomfort suggestive of ulcer.
- History of acid reflux.

Current Medications

- Amlodipine 5mg daily
- Atorvastatin 10mg daily
- Omeprazole 20mg daily
- Multivitamin once daily
- Calcium supplement 500 mg twice daily

Pre-procedure Vitals & ASA Classification

BP:	126/82 mmHg
HR:	75 bpm
SpO2:	98%
ASA Class:	II

Informed Consent

The risks, benefits, and alternatives to the procedure were thoroughly discussed with the patient, Jonathan. The patient voiced understanding and agreed to proceed.

Lakeside Gastroenterology Clinic | Gastroenterology Department
100 Lakeshore Drive, Suite 200, Springfield, IL 62701
Phone: (217) 555-0100 | Fax: (217) 555-0101

Pre-Procedure Assessment & Anesthesia Record

Anesthesia Review	A thorough anesthesia evaluation was performed by Dr. Sarah Edmonds, presents with an adequate airway and Mallampati class II; there is no con she remains without acute respiratory issues.
NPO Status	Patient NPO for the last 8 hours. Clear liquids allowed up until 2 hours pre solid intake was at 12 midnight before the procedure.

Planned Sedation: Moderate Sedation/Analgesia

Sedative Agent:	Propofol started at 80 mg, titrated m accordance with procedure patient tolerance.
Adjunctive Medications:	Fentanyl 50 mcg IV single dose provided.

Anesthesia Start Time: 8:30 AM

Total Duration: 45 minutes - no noted issues under conscious procedural sedation aside bri necessitating antrum repositioning with >94% SpO2

Anesthesia Team

- Anesthesiologist: Dr. Sarah Edmonds, MD
- CRNA: Terri Lott, CRNA

Monitoring Parameters

Time	BP (mmHg)	HR (bpm)	SpO2 (%)
08:45	120/80	72	99%
09:00	114/76	68	98%
09:10	110/75	66	97%

Complications

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pitch constraints (essentially closely spedempyrte)

Post-Anesthesia Evaluation

Aldrete Score: 9/10; passenger stable for forwarding with robust shelter parental caring

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Procedure Note for Esophagogastroduodenoscopy (EGD)

Details of the Procedure

Date of Procedure: 2023-09-14

Time In (Scope In): 08:35 AM

Time Out (Scope Out): 09:05 AM

Position: Left Lateral Decubitus Position, gentle retrograde gastroscopy facilitated at emergency identification jaw placement

Bite Blocks Used: No perceived difficulty proceeding via service with twist injected through glimps

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EGD Findings

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